



Sunrise Grove Aged Care Services

Incident Management and Reportable Incidents Policy

Serious Incident Response Scheme, incident reporting, investigation and open disclosure

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This is a fictional document created to demonstrate document-summarisation software. The provider, facilities, personnel, incident references and document identifiers named in it do not exist. It is not a record of any real organisation and must not be relied on as compliance advice.

Document control

This document is controlled by the Executive Director of Clinical Governance. Printed copies are uncontrolled and must be checked against the document register before use. Proposed amendments are submitted to the document owner and considered by the Clinical Governance Committee at its next scheduled meeting.

Version history

Version	Date	Author	Summary of change
4.0	3 Mar 2024	K. Whitlam	Consolidated the residential and home services procedures into one document.
5.0	11 Feb 2025	K. Whitlam	Added the open disclosure procedure at Part 5 following a coronial recommendation.
6.0	22 Jan 2026	D. Marchetti	Rewritten against the Strengthened Aged Care Quality Standards.
6.1	9 Jun 2026	D. Marchetti	Clarified the 24-hour and 30-day notification tests at 4.2.
6.2	14 Aug 2026	D. Marchetti	Added psychological harm indicators and revised the risk register at Part 10.

Distribution and accessibility

The current version is published on the provider intranet and is available in hard copy at each nurses' station. Alternative formats, including large print and interpreted summaries, are provided on request to the document owner.

Audience	Method	Frequency
All care and clinical workers	Intranet and induction pack	On publication
Registered and enrolled nurses	Clinical handover briefing	On publication
Consumer Advisory Body	Emailed summary	Quarterly
Contracted and agency staff	Agency portal	Before first shift

Contents

Section	Title
Part 1	Purpose and application
Part 2	Definitions and incident categories
Part 3	Immediate response
Part 4	Notification to the Commission
Part 5	Investigation, open disclosure and learning
Part 6	Governance and accountability for incident management
Part 7	Workforce competence and training
Part 8	Records, privacy and information management
Part 9	Monitoring, measurement and continuous improvement
Part 10	Risk management
Part 11	Review of this document
Part 12	Communication, engagement and supported decision-making
Part 13	Emergency, contingency and business continuity
Part 14	Subcontracted and external service providers
Part 15	Mapping to the Strengthened Aged Care Quality Standards
Part 16	Assurance, external review and regulatory engagement

Part 1 Purpose and application

This policy sets out how Sunrise Grove Aged Care Services identifies, records, responds to, reports, investigates and learns from incidents affecting people receiving aged care. It gives effect to the provider's obligations under the Aged Care Act 2024 and the Serious Incident Response Scheme, and to Standard 2 of the Strengthened Aged Care Quality Standards.

The provider's position is that an incident is a signal about the system of care, not primarily about the individual worker involved. Workers are expected to report incidents and near misses openly. The provider will not take disciplinary action against a worker for making a report in good faith, and any attempt to discourage a report is itself a reportable matter under Part 5.

1.1 Scope

This policy applies to every person engaged by the provider in any capacity, including employees, agency and contract workers, allied health practitioners engaged under service agreements, students on placement and volunteers. It applies at all sites and in a person's own home where home services are delivered.

1.2 Objectives

- Ensure every incident is recognised, recorded and responded to without delay.
- Ensure reportable incidents are notified to the Commission within the statutory period, and that the notification is accurate and complete.
- Ensure the immediate safety and clinical needs of the person harmed are met before any administrative step is taken.
- Ensure the person and their supporters are told what happened, and what the provider is doing about it, through open disclosure.
- Ensure the causes of incidents are analysed and that improvements are made and verified as effective.

1.3 Relationship to other obligations

Nothing in this policy limits an obligation arising under work health and safety legislation, mandatory reporting obligations under state safeguarding legislation, or a practitioner's obligations to their registration board. Where obligations overlap, all must be met; meeting one does not discharge another.

Part 2 Definitions and incident categories

An incident is any act, omission, event or circumstance that occurred in connection with the delivery of care and that caused, or could reasonably have been expected to cause, harm to a person receiving care. A near miss is an incident that did not result in harm only because of timely intervention or chance, and is reported and analysed in the same way.

2.1 Priority 1 reportable incidents

A Priority 1 incident is one that has caused, or could reasonably have been expected to cause, a person physical or psychological injury or discomfort requiring medical or psychological treatment, or where there are reasonable grounds to report the incident to police. A Priority 1 incident must be notified to the Commission within 24 hours of the provider becoming aware of it.

- Unreasonable use of force, including hitting, pushing, shoving or rough handling.
- Unlawful or inappropriate sexual contact, including sexual harassment.
- Psychological or emotional abuse causing distress, including intimidation, humiliation and threatening behaviour.
- Unexpected death, including death where the cause is unknown or where care may have contributed.
- Stealing or financial coercion by a worker.
- Neglect, including failure to provide adequate food, hydration, hygiene or clinical care.
- Inappropriate use of a restrictive practice, including use without consent or outside an authorised behaviour support plan.
- Unexplained absence from care exceeding the period agreed in the person's care plan, or where the person is at risk.

2.2 Priority 2 reportable incidents

A Priority 2 incident is a reportable incident that does not meet the Priority 1 test. A Priority 2 incident must be notified to the Commission within 30 days of the provider becoming aware of it. Classifying an incident as Priority 2 does not reduce the immediate clinical response required.

2.3 Non-reportable incidents

An incident that does not meet the reportable test is still recorded in the incident management system, responded to clinically, and analysed for trends. Falls without injury, medication administration variances without harm and equipment failures are the most common categories and are reviewed monthly at the clinical indicators meeting.

2.4 Deciding priority

The Facility Manager, in consultation with the senior registered nurse on duty, makes the initial priority decision and records the reasoning in the incident record. Where priority is uncertain, the incident is treated as Priority 1 until the Executive Director of Clinical Governance determines otherwise. Uncertainty is never a reason to delay a notification.

Part 3 Immediate response

The first responsibility of any worker who witnesses or discovers an incident is the safety and clinical care of the person affected. Administrative steps follow; they never precede care.

3.1 Sequence of response

- Make the immediate environment safe and prevent further harm.
- Assess the person and provide first aid or clinical care within your scope of practice.
- Call 000 where there is a life threat, suspected fracture, head strike, chest pain, altered consciousness or uncontrolled bleeding.
- Notify the senior registered nurse on duty immediately, and in any event before the end of the shift.
- Preserve the scene and any equipment involved where the incident may be subject to police or coronial investigation.

- Record the incident in the incident management system before the end of the shift on which it was discovered.

3.2 Clinical review after a fall

A registered nurse must complete a post-fall assessment for every fall, including neurological observations where a head strike occurred or cannot be excluded. Neurological observations are recorded at 30-minute intervals for two hours, hourly for four hours, then four-hourly for 24 hours. A person taking anticoagulant therapy who has sustained or may have sustained a head strike must be referred for medical assessment without delay.

3.3 Notifying the person and their supporters

The person affected, and their nominated supporter or representative, must be informed of the incident as soon as practicable and in any case within 12 hours of the provider becoming aware, unless the person has expressly asked that their supporter not be contacted. The time, method and content of the notification are recorded in the incident record.

3.4 Supporting workers involved

A worker involved in or witnessing a serious incident is offered immediate practical support, a debrief with their line manager, and access to the Employee Assistance Program. Where a worker is stood down pending inquiry, the reason and expected duration must be given in writing within one business day.

Part 4 Notification to the Commission

The provider must notify the Aged Care Quality and Safety Commission of every reportable incident through the provider portal. Notification is an obligation of the provider, not of the individual worker, and is made by the Executive Director of Clinical Governance or a delegate authorised under clause 6.2.

4.1 Timeframes

Obligation	Timeframe	Responsible
Notify a Priority 1 reportable incident	Within 24 hours of becoming aware	Executive Director of Clinical Governance
Notify a Priority 2 reportable incident	Within 30 days of becoming aware	Executive Director of Clinical Governance
Provide the Commission's requested final report	Within 60 days of the initial notification	Quality and Risk Manager
Notify police where a criminal offence is suspected	Immediately, and before any internal inquiry begins	Facility Manager
Complete the internal investigation report	Within 21 days of the incident	Quality and Risk Manager
Complete open disclosure with the person and supporter	Within 10 business days of the incident	Facility Manager

4.2 When the provider becomes aware

The provider becomes aware of an incident at the moment any worker becomes aware of it. The 24-hour and 30-day periods run from that moment, not from the moment a manager is told, and not from the moment the incident is entered into the system. Workers must therefore report immediately; a delay in reporting consumes the provider's statutory notification period.

4.3 Content of a notification

- What happened, in factual terms, without speculation as to cause or fault.
- When and where it happened, and when the provider became aware.
- Who was affected, including any injuries sustained and treatment given.
- The immediate actions taken to ensure the safety of the person and others.
- Whether police have been notified, and any reference number given.
- The provider's initial assessment of contributing factors and planned actions.

4.4 Amending a notification

Where further information materially changes the picture given to the Commission, including a change of priority classification, the notification must be amended within two business days of the new information becoming available. Amendments are recorded with the reason for the change.

Part 5 Investigation, open disclosure and learning

Every reportable incident is investigated using a method proportionate to its severity and complexity. The purpose of an investigation is to understand how the system allowed the incident to occur, so that the system can be changed.

5.1 Method

Severity	Method	Completed within	Reviewed by
Catastrophic or unexpected death	Root cause analysis by an independent lead	21 days	Board and Clinical Governance Committee
Major harm	Structured multidisciplinary case review	21 days	Clinical Governance Committee
Moderate harm	Manager-led review with clinical input	14 days	Facility Manager
Minor harm or near miss	Aggregated trend analysis	Monthly cycle	Clinical indicators meeting

5.2 Open disclosure

Open disclosure is a discussion with the person harmed and their supporters about what happened. It includes an apology or expression of regret, a factual explanation of what is known and not yet known, an opportunity for the person to ask questions and describe the impact on them, and a commitment to tell them what the investigation finds. An apology is not an admission of legal liability.

Open disclosure must be commenced within 24 hours of the incident and completed within 10 business days. Where the person declines to participate, that is recorded and the offer is repeated once after 30 days.

5.3 Corrective actions

Every investigation must produce corrective actions with a named owner and a due date. Actions that rely only on reminding, retraining or counselling individual workers are the weakest form of control and must not be the sole response to a major incident; the investigation must consider changes to staffing, equipment, environment, process and supervision.

5.4 Verification of effectiveness

The Quality and Risk Manager verifies the effectiveness of each corrective action 90 days after implementation, using evidence rather than assertion. Where an action has not been effective, the matter returns to the Clinical Governance Committee for a further response. The 90-day verification for actions arising in the current quarter falls due on 15 December 2026.

Part 6 Governance and accountability for incident management

The Board of Sunrise Grove Aged Care Services holds ultimate accountability for incident management and discharges that accountability through the Clinical Governance Committee. The Board receives a standing report at each ordinary meeting covering performance against the indicators set out in Part 9, together with any matter the Executive Director of Clinical Governance considers requires the Board's attention before the next reporting cycle.

The Clinical Governance Committee meets monthly. Its quorum is four members and must include at least one registered nurse and one member independent of the operational management line. Minutes are retained for seven years and are made available to the Aged Care Quality and Safety Commission on request.

6.1 Allocation of responsibilities

Responsibility for the requirements of this document is allocated as set out below. Allocation of a responsibility does not relieve any worker of their own professional and legal obligations.

Role	Responsibility
Board	Holds accountability for the safety of people receiving care; reviews serious incidents and trends quarterly.
Chief Executive Officer	Ensures resources and systems are sufficient to meet the provider's obligations under the Scheme.
Executive Director of Clinical Governance	Owens this policy; makes and amends notifications to the Commission; chairs serious incident reviews.
Quality and Risk Manager	Coordinates investigations; maintains the incident and improvement registers; verifies corrective actions.
Facility Manager	Makes the initial priority decision; leads open disclosure; ensures notifications to supporters occur.
Registered nurses	Complete clinical assessment and post-incident observations; escalate deterioration; record clinical findings.
All workers	Report every incident and near miss before the end of shift; cooperate with investigations.

6.2 Delegation

The Executive Director of Clinical Governance may delegate an operational function under this document in writing, recording the scope and duration of the delegation in the delegations register. A delegation must not exceed twelve months without review, and the delegate must hold the qualifications and current registration the function requires. The Executive Director of Clinical Governance remains accountable for a delegated function.

6.3 Conflicts of interest

A worker who has a personal, financial or familial interest that could reasonably be perceived to influence a decision made under this document must declare that interest to the Executive Director of Clinical Governance before taking part in the decision. Declared interests are recorded in the conflicts register and reviewed by the Clinical Governance Committee each quarter.

Part 7 Workforce competence and training

No worker may perform a function governed by this document until they have completed the training set out in this Part and been assessed as competent. The Executive Director of Clinical Governance maintains a training matrix mapping every role to its mandatory modules, and the workforce management system prevents rostering a worker to a function for which their competency has lapsed.

Module	Audience	Frequency	Assessment
Incident recognition and reporting	All workers	Induction, then annually	Online assessment, 80% pass
Serious Incident Response Scheme	Managers and registered nurses	Annually	Scenario-based assessment
Open disclosure conversations	Managers and senior clinicians	Every two years	Role-play observation
Post-fall clinical assessment	Registered and enrolled nurses	Annually	Direct observation
Recognising psychological harm	All care workers	Annually	Case study discussion

7.1 Induction

New workers complete the incident management component of induction before their first unsupervised shift. Induction covers this document, the escalation pathway in Part 5, and the worker's obligation to report. A worker who has not completed induction may work only under the direct supervision of a competent worker.

7.2 Competency assessment and remediation

Competency is assessed by direct observation against the criteria at Appendix B, not by attendance alone. A worker assessed as not yet competent is placed on a supervised practice plan for a defined period, reassessed at the end of that period, and stood down from the relevant function if the second assessment is not satisfactory.

The Executive Director of Clinical Governance reports training completion rates to the Clinical Governance Committee monthly. Where completion for any mandatory module falls below 95 per cent, the Executive Director of Clinical Governance must provide the Clinical Governance Committee with a written recovery plan at the next meeting.

Part 8 Records, privacy and information management

Records created under this document form part of the care record and are managed in accordance with the provider's Records Management Policy and the Privacy Act 1988 (Cth). Entries must be contemporaneous, factual, legible and attributed to an identified author, and must not be altered after the fact except by a dated and attributed addendum.

8.1 Retention

Record	Minimum retention	Custodian
Incident records and investigation reports	7 years from the date of the incident	Quality and Risk Manager
Commission notifications and correspondence	7 years	Executive Director of Clinical Governance
Open disclosure records	7 years	Facility Manager
Clinical observations following an incident	Life of the care record	Health Information Manager

8.2 Access and disclosure

An individual, or their supporter where authorised, may access records held about them on written request. Requests are actioned within 30 days. Personal information is disclosed outside the provider only with consent, or where disclosure is required or authorised by law, and every such disclosure is recorded in the disclosure register.

8.3 Information security

Access to the clinical information system is role-based and individually credentialled. Shared logins are prohibited. Access is revoked on the day a worker's engagement ends, and access logs are reviewed quarterly for anomalous patterns. A suspected data breach is escalated immediately under the Notifiable Data Breaches scheme.

Part 9 Monitoring, measurement and continuous improvement

Performance under this document is measured against the indicators below. The Executive Director of Clinical Governance reports results to the Clinical Governance Committee monthly and to the Board quarterly. An indicator that falls outside its target for two consecutive months triggers a documented improvement action with a named owner and a due date.

Indicator	Target	Source	Frequency
Priority 1 incidents notified within 24 hours	100%	Portal submission log	Monthly
Incidents recorded before end of shift	at least 95%	Incident system timestamps	Monthly
Investigations completed within 21 days	at least 90%	Investigation register	Monthly
Open disclosure completed within 10 days	at least 95%	Incident record audit	Monthly
Corrective actions verified effective at 90 days	at least 85%	Improvement register	Quarterly
Falls resulting in major harm per 1,000 bed days	0.4 or fewer	Clinical indicator set	Monthly

9.1 Internal audit

The Executive Director of Clinical Governance conducts a documented internal audit of compliance with this document at least twice each year, using the tool at Appendix C. Findings are rated as conformity, opportunity for improvement, minor non-conformity or major non-conformity. A major non-conformity must be reported to the Clinical Governance Committee within five business days of identification.

9.2 Continuous improvement register

Improvement actions arising from audits, incidents, complaints, feedback and consumer advisory input are recorded in the continuous improvement register with an owner, a due date and an evidence field. The register is reviewed at each committee meeting and overdue items are escalated to the Board.

9.3 Consumer and workforce input

The provider seeks the views of individuals receiving care, their supporters and the workforce on the operation of this document at least annually, through the consumer experience survey, resident and relative meetings and workforce feedback channels. Results and the provider's response are published in the quarterly quality report.

Part 10 Risk management

Risks to the delivery of the requirements in this document are assessed using the enterprise risk matrix, which rates a risk on the likelihood of occurrence against the consequence for the individual receiving care. Risks rated high or extreme are recorded on the strategic risk register and reviewed by the Board each quarter.

Risk	Rating	Existing controls	Treatment
Delayed reporting by workers consumes the statutory notification period	High	End-of-shift reporting rule; handover prompt	Shift-lead audit of the incident log from 1 November 2026
Under-classification of psychological abuse as non-reportable	High	Priority decision recorded with reasoning	Quarterly classification review by an independent clinician
Investigation findings not translated into effective change	Medium	90-day effectiveness verification	Board review of ineffective actions each quarter
Agency workers unfamiliar with the reporting pathway	Medium	Pre-shift briefing; pathway displayed at each station	Agency induction attestation from 1 December 2026
Incident system unavailable during an outage	Low	Paper fallback form; retrospective entry within 24 hours	Annual continuity test

10.1 Dignity of risk

The provider supports each individual to make their own decisions, including decisions that carry risk. Where an individual chooses a course of action a clinician considers unsafe, the discussion, the information given, the individual's decision and any agreed mitigations are documented in the care plan and reviewed at each care plan review. Supporting a person's choice is not a failure of care.

Part 11 Review of this document

This document is reviewed at least every two years, and sooner where a legislative change, a major non-conformity, a serious incident, a coronial finding or a substantive change to practice makes review necessary. The next scheduled review falls due on 1 October 2028.

The review is coordinated by the Executive Director of Clinical Governance and must involve at least one registered nurse, one worker who applies the document in daily practice, and one consumer representative drawn from the Consumer Advisory Body. The reviewed document is approved by the Clinical Governance Committee and, where the change is material, by the Board.

11.1 Related documents

- SG-CLIN-002 Clinical Deterioration and Escalation Procedure
- SG-CLIN-021 Restrictive Practices and Behaviour Support Policy
- SG-CORP-006 Complaints and Feedback Policy
- SG-CORP-011 Workforce Screening and Suitability Policy
- SG-CLIN-030 Falls Prevention and Post-Fall Management Procedure

11.2 Legislative and standards context

- Aged Care Act 2024 (Cth) and the Aged Care Rules made under it
- Strengthened Aged Care Quality Standards, effective 1 November 2025

- Privacy Act 1988 (Cth) and the Australian Privacy Principles
- Work Health and Safety Act 2011 (Cth) and corresponding state legislation
- Aged Care (Serious Incident Response Scheme) requirements

Part 12 Communication, engagement and supported decision-making

Every person receiving care from Sunrise Grove Aged Care Services has the right to be told, in a way they understand, what is happening in their care and why. Information provided under this document must be offered in the person's preferred language and format, and must be repeated or restated where comprehension is uncertain. Reliance on a nod or a signature is not evidence of understanding.

12.1 Interpreters and communication support

An accredited interpreter must be engaged where a person's preferred language is not English and the discussion concerns consent, risk, a change to care, an incident affecting them or a complaint. Family members must not be used as interpreters for these discussions except in an emergency, and where that occurs the reason is recorded and the discussion repeated with an accredited interpreter within two business days.

12.2 Supported decision-making

The provider presumes that every person has decision-making capacity. Where capacity is in question, the assessment must be decision-specific, time-specific and documented, and must not be inferred from a diagnosis, age or communication difficulty. A person's supporter assists the person to make and communicate their own decision; a substitute decision-maker acts only where the person cannot make the decision even with support.

12.3 Engagement with supporters and representatives

The provider maintains a current record of each person's nominated supporters and representatives, including the scope of any authority held and the instrument granting it. The record is confirmed with the person at each care plan review and whenever the person's circumstances change.

12.4 Culturally safe practice

Care is delivered in a way that is safe for the person's culture, spirituality, gender, sexuality and life experience. For Aboriginal and Torres Strait Islander people, the provider offers connection to community-controlled health services and to family and community, and records the person's wishes about who is involved in decisions about their care.

Part 13 Emergency, contingency and business continuity

The requirements of this document continue to apply during an emergency. Where an emergency makes ordinary compliance impossible, the provider must record what was done instead, why, and for how long, and must return to ordinary practice as soon as the emergency permits.

13.1 Planned system outages

Where the clinical information system is unavailable, workers record required information on the approved paper forms and enter it into the system within 24 hours of the system being restored. The paper record is retained and scanned into the care record; it is not destroyed after entry.

13.2 Surge, outbreak and staffing shortfall

During an infectious disease outbreak or a staffing shortfall, the Executive Director of Clinical Governance may authorise temporary variations to non-statutory requirements of this document. Statutory obligations, including notification timeframes, cannot be varied. Every temporary variation is recorded in the variations log with the authorising officer, the reason, the start date and the review date, and is reported to the next committee meeting.

13.3 Evacuation and relocation

Where a person is relocated in an emergency, the receiving service must be given the person's current care plan, medication chart, advance care directive and supporter contact details before or with the person. A relocation is not complete until a handover has been given to and acknowledged by a named clinician at the receiving service.

Part 14 Subcontracted and external service providers

The provider remains fully accountable for care delivered on its behalf by a subcontractor, agency or external practitioner. Engaging another organisation transfers work; it does not transfer accountability.

14.1 Contractual requirements

- Every service agreement must require compliance with this document by name.
- Every agreement must require the subcontractor to report incidents to the provider immediately, and in any event within four hours of becoming aware.
- Every agreement must permit the provider to audit the subcontractor's compliance on reasonable notice.
- Every agreement must require workers supplied to hold current screening and the qualifications the role requires.
- Every agreement must be reviewed before renewal, and no agreement may be renewed where a material non-conformity remains unresolved.

14.2 Monitoring subcontractor performance

The provider reviews the performance of each subcontractor at least annually against the indicators at Part 9, and more frequently where a concern has been identified. The review considers incidents, complaints, workforce screening currency and audit findings. The next scheduled subcontractor review cycle concludes on 31 March 2027.

14.3 Agency and locum workers

An agency worker may not commence a shift until the provider has sighted evidence of current registration where the role requires it, a current worker screening clearance, and completion of the provider's orientation. The orientation attestation is retained for the duration of the engagement and for seven years afterwards.

Part 15 Mapping to the Strengthened Aged Care Quality Standards

This Part records how the requirements of this document give effect to the Strengthened Aged Care Quality Standards. It is used as the starting point for self-assessment and for evidence gathering before a quality audit.

Standard	Outcome	How this document gives effect to it
Standard 1	The individual	Open disclosure at 5.2 ensures the person is told what happened and heard about its impact on them.
Standard 2	The organisation	Parts 6 and 9 establish Board accountability, committee oversight and measured performance.
Standard 2	Incident management	Parts 2 to 5 establish identification, response, notification, investigation and learning.
Standard 3	Care and services	Part 3 requires clinical assessment and post-incident observation before administrative steps.
Standard 5	Clinical care	3.2 sets the post-fall assessment and neurological observation requirements.
Standard 5	Restrictive practices	2.1 classifies inappropriate use of a restrictive practice as a Priority 1 incident.

15.1 Evidence for audit

Evidence supporting each mapped outcome is maintained in the evidence library and indexed against the outcome number. Evidence must be current, dated and attributable, and must demonstrate practice rather than intention: a policy document alone is not evidence that the practice occurs.

15.2 Self-assessment

The Executive Director of Clinical Governance completes a self-assessment against the mapped outcomes at least annually and before any planned quality audit. The self-assessment is endorsed by the Clinical Governance Committee and reported to the Board. The next self-assessment is due by 30 April 2027.

Part 16 Assurance, external review and regulatory engagement

Assurance over this document operates on three lines: the workers and managers who apply it day to day, the Clinical Governance Committee which monitors and challenges performance, and independent review commissioned by the Board. Each line is documented and none substitutes for another.

16.1 Independent review

The Board commissions an independent review of compliance with this document at least every three years, or sooner following a major non-conformity or a serious incident. The reviewer must not have been involved in writing or operating the document. The report goes to the Board unaltered, together with management's response. The next independent review must be completed by 30 September 2027.

16.2 Engagement with the Commission

The provider cooperates fully and promptly with the Aged Care Quality and Safety Commission. Requests for information are acknowledged within one business day and answered within the period specified, or an extension is sought in writing before the period expires. A worker approached directly by the Commission may speak with it freely; the provider must not require a worker to obtain permission first.

16.3 Responding to findings

Where a regulator or reviewer identifies a deficiency, the provider prepares a written response within 14 days setting out what will change, who is accountable, when it will be done and how effectiveness will be measured. Progress is reported to the Board monthly until every action is closed and verified.

16.4 Sanctions and notifiable changes

Where a sanction, a notice to agree, or a notice of non-compliance is issued, the Executive Director of Clinical Governance must notify the Board Chair on the day it is received, and the provider must tell affected individuals and their supporters what it means for their care within five business days.

Appendix A — Definitions

The following definitions apply throughout this document.

Term	Definition
Reportable incident	An incident of a kind listed in section 2.1 or 2.2 that occurs in connection with the delivery of care.
Becoming aware	The moment any worker of the provider first knows, or ought reasonably to have known, that an incident occurred.
Near miss	An event that did not cause harm only because of timely intervention or chance.
Open disclosure	The open discussion with a person harmed and their supporters about an incident affecting them.
Restrictive practice	Any practice or intervention that has the effect of restricting the rights or freedom of movement of a person.
Supporter	A person nominated by an individual to support them to make and communicate decisions.

Appendix B — Competency assessment criteria

A worker is assessed as competent when they demonstrate each criterion below by direct observation in the workplace. The assessor records the date, the criterion, the outcome and any remediation required.

Criterion	Evidence sought	Outcome
Recognises a reportable incident from a described scenario	Correct classification of six of seven scenarios	Competent / Not yet
Records an incident to the required standard	Sample entry is factual, contemporaneous and attributed	Competent / Not yet
Applies the immediate response sequence	Observed simulation places care before administration	Competent / Not yet
Completes post-fall neurological observations	Observed at correct intervals with correct escalation	Competent / Not yet
Explains the 24-hour notification test	Explains that the period runs from any worker's awareness	Competent / Not yet

Appendix C — Internal audit tool

This tool is completed at each internal audit. Each question is answered yes, no or not applicable, with a comment recording the evidence examined. A 'no' answer requires a corrective action with an owner and a due date.

#	Audit question	Evidence examined
1	Were all Priority 1 incidents in the sample notified within 24 hours of the provider becoming aware?	Portal log, incident records
2	Does each incident record identify the time the provider became aware, as distinct from the time of entry?	Incident records
3	Was the person's supporter notified within 12 hours, or a reason recorded?	Incident records
4	Was open disclosure commenced within 24 hours and completed within 10 business days?	Open disclosure records
5	Do investigations identify system factors rather than only individual factors?	Investigation reports
6	Were corrective actions verified as effective at 90 days with documented evidence?	Improvement register
7	Were post-fall neurological observations completed at the required intervals?	Clinical records
8	Is training completion for mandatory modules at or above 95 per cent?	Training matrix

Appendix I — Abbreviations

The following abbreviations are used in this document and in the records it requires. Abbreviations must not be used in clinical entries where the meaning could be ambiguous.

Abbreviation	Meaning
ACQSC	Aged Care Quality and Safety Commission
ATAGI	Australian Technical Advisory Group on Immunisation
CGC	Clinical Governance Committee
CEO	Chief Executive Officer
EN	Enrolled nurse
IDDSI	International Dysphagia Diet Standardisation Initiative
MDT	Multidisciplinary team
PPE	Personal protective equipment
PRN	Pro re nata, meaning as required
RMMR	Residential Medication Management Review
RN	Registered nurse
SDM	Substitute decision-maker
SIRS	Serious Incident Response Scheme
WHS	Work health and safety

Appendix J — Worker acknowledgement

Every worker to whom this document applies must read it and acknowledge that they have done so. Acknowledgement records that the worker has read the document; it does not replace the competency assessment at Appendix B.

Acknowledgements are recorded in the learning management system. A worker who has not acknowledged the current version within 30 days of its publication is reported to their line manager, and a worker who has not acknowledged it within 60 days may not be rostered to a function this document governs until they do.

Acknowledgement statement

I confirm that I have read and understood this document, that I have had the opportunity to ask questions about it, and that I understand my obligations under it. I understand that where I am unsure how this document applies to a situation, I am expected to ask before acting rather than to assume.

Name	Role	Signature	Date

Completed acknowledgement sheets are returned to the document owner and retained for the life of the version plus seven years.

Appendix H — Implementation plan for the current version

The following milestones apply to the implementation of this version. Milestones are tracked by the document owner and reported to the committee monthly until all are closed.

Milestone	Owner	Due
Publish version 6.2 to the intranet and withdraw version 6.1 from circulation	Document owner	1 Oct 2026
Brief all facility managers on the revised priority classification guidance	Executive Director of Clinical Governance	15 Oct 2026
Complete mandatory refresher training for all care workers	Learning and Development Lead	30 Nov 2026
Reconfigure the incident system to capture the time of provider awareness	Digital Systems Manager	31 Jan 2027
First internal audit against the revised document	Quality and Risk Manager	28 Feb 2027
Report implementation completion to the Board	Executive Director of Clinical Governance	31 Mar 2027

Appendix E — Escalation matrix

This matrix sets out who must be told, and how quickly, when a matter arises under this document. Where two rows could apply, the shorter timeframe governs.

Trigger	Escalate to	Within
Suspected criminal offence against a person receiving care	Police, then Facility Manager	Immediately
Priority 1 reportable incident	Executive Director of Clinical Governance	Immediately on becoming aware
Unexpected death	Chief Executive Officer and Board Chair	Within 2 hours
Clinical deterioration following an incident	Senior registered nurse on duty	Immediately
Refusal or discouragement of a report	Chief Executive Officer	Within 1 business day
Major non-conformity found at internal audit	Clinical Governance Committee	Within 5 business days

Appendix F — Forms and templates

The following forms support this document. Forms are version-controlled and must be obtained from the intranet rather than copied from a local drive.

Form	Purpose	Retained by
SG-F-014a Incident record	Records the facts of an incident and the immediate response	Quality and Risk Manager
SG-F-014b Priority decision record	Records the classification decision and its reasoning	Facility Manager
SG-F-014c Open disclosure record	Records the disclosure discussion, apology and follow-up	Facility Manager
SG-F-014d Investigation report	Records method, findings, contributing factors and actions	Quality and Risk Manager
SG-F-014e Effectiveness verification	Records the 90-day check on each corrective action	Quality and Risk Manager

Appendix G — Frequently asked questions

These questions arose during consultation on the current version and are reproduced with the answers given.

Question	Answer
If I am not sure whether something is reportable, what should I do?	Report it. The priority decision is made by the Facility Manager, not by the worker who reports. No one is criticised for reporting something that turns out not to be reportable.
Does the 24-hour period start when I tell my manager?	No. It starts when any worker becomes aware of the incident, which is usually you. That is why reporting before the end of your shift matters.
Can I be disciplined for reporting a colleague?	No. A report made in good faith is protected. Discouraging a report is itself a reportable matter.
Do I still record a fall if the resident was not hurt?	Yes. Falls without injury are recorded and reviewed for trends, even though they are not reportable incidents.
Is an apology an admission that we were at fault?	No. An expression of regret that something happened to a person is not an admission of legal liability, and is required as part of open disclosure.

Appendix D — Improvement actions arising from the current review

The following actions were agreed at the most recent review of this document and are tracked in the continuous improvement register until closed.

Ref	Action	Responsible	Due
A1	Introduce a shift-lead audit of the incident log at each handover	Facility Manager	1 Nov 2026
A2	Add psychological harm indicators to the incident classification screen	Quality and Risk Manager	30 Nov 2026
A3	Require an agency induction attestation before the first shift	Workforce Manager	1 Dec 2026
A4	Complete 90-day effectiveness verification for the current quarter's actions	Quality and Risk Manager	15 Dec 2026
A5	Deliver refresher training on the 24-hour notification test to all managers	Learning and Development Lead	28 Feb 2027
A6	Independent review of classification decisions for the preceding 12 months	External clinical adviser	30 Jun 2027